shortage of components which it was importing too.[9] With a ban on flights there had been disruptions in the supply of vital components across product categories around the world of which China and India were no exceptions. Media reports were emerging from various countries like the UK, EU, US and Brazil about acute ventilator shortage and an inability to procure them.[10] [11] After China's rejection, India immediately placed orders with international companies like Hamilton, Mindray and Draeger for urgent sourcing of units.[12] However, relying on these for our bulk, long-term requirement was not feasible. The empowered group for ensuring availability and production of essential medical equipment began to work towards creating solutions for mass supply, which could be completely manufactured in India. Keeping in mind that COVID-19 primarily affects pulmonary functions, the DRDO programme, Society for Biomedical Technology, began efforts to find innovative solutions to create a 'multi-patient' ventilator. After achieving a working prototype with collective effort by the public–private effort of the DRDO and M/s Skanray and Bharat Electronics Limited (BEL), nine companies were identified for design transfer so that manufacturing could be scaled up. The DRDO zeroed down on Skanray to produce critical care ventilators. At the same time, the DRDO began to make efforts towards indigenizing components which would be required to produce these prototyped devices. Eventually, they were able to achieve 70–80 per cent domestic production of components which went into these devices. Two public-sector companies—BEL enabled by Skanray and the Andhra Pradesh MedTech Zone—committed to the GoI that they would make 30,000 and 13,000 devices, respectively, within two months.

Businessman Anand Mahindra, the head of Mahindra and Mahindra group, had indicated early on in March about how the Mahindra Group would immediately begin brainstorming on how their automobile manufacturing facilities could make ventilators.[13] Skanray had the capacity of building only 5,000 ventilators in three months. In order to augment its scale to produce 30,000 units in two months, it needed to tie up with larger players such as Mahindra and Mahindra. To reduce import and development time, automotive components used in Mahindra vehicles were selected to produce a simpler version of the full-fledged mechanical ventilator used in ICUs.